Vet. App. No. 19-1814

IN THE UNITED STATES COURT OF APPEALS
FOR VETERANS CLAIMS

RALPH PENNEY
Appellant,

v.

ROBERT L. WILKIE,
Secretary of Veterans Affairs,
Appellee.

ON APPEAL FROM THE BOARD OF VETERANS' APPEALS

BRIEF OF APPELLEE SECRETARY OF VETERANS AFFAIRS

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v.	)	Vet. App. No. 19-1814
ROBERT L. WILKIE, Secretary of Veterans Affairs, Appellee.	)	

ON APPEAL FROM THE BOARD OF VETERANS' APPEALS

BRIEF OF APPELLEE SECRETARY OF VETERANS AFFAIRS

ISSUES PRESENTED

Whether the Court should affirm the Board of Veterans' Appeals' (Board) February 15, 2019, decision that denied a total disability rating based on individual unemployability due to service-connected disabilities (TDIU) for the period before January 21, 2010, when the Board correctly applied the law, did not clearly err in any of its factual findings, and provided an adequate statement of reasons or bases.

STATEMENT OF THE CASE

A. JURISDICTIONAL STATEMENT

The Court has exclusive jurisdiction to review the final decisions of the Board under 38 U.S.C. § 7252(a).

B. NATURE OF THE CASE

On February 15, 2019, the Board issued a decision denying TDIU before January 21, 2010. (Record Before the Agency (R.) at 5 (5-18)). Appellant, Ralph

Penney, appealed to this Court in March 2019.

C. STATEMENT OF RELEVANT FACTS

Private treatment records from September 2008 document that Appellant had coronary artery disease and cerebrovascular disease. (R. at 2181). The private physician noted that Appellant had undergone a coronary artery bypass grafting that caused a major improvement in his symptoms. (*Id.*). The physician further noted that Appellant had no neurologic symptoms and no angina. (*Id.*). Three months later, in December 2008, Appellant sought service connection for diabetes. (R. at 2378 (2370-83)). In that same application, Appellant also sought service connection for a heart condition, high blood pressure, glaucoma, and post-traumatic stress disorder (PTSD). (*Id.*). In February 2009, Appellant attended a VA heart examination, during which he reported a history of a myocardial infarction in 2004, as well as a history of shortness of breath. (R. at 859 (858-61)). In that regard, Appellant stated that he could walk about 20 minutes on flat ground without getting any shortness of breath but that he would get short of breath if he walked up hills. (R. at 860). Appellant denied any history of chest pain or tightness. (*Id.*). That same day, Appellant also underwent a VA diabetes examination. (R. at 853-58). The examiner noted that the onset of that condition was in June 2008 and that the side effect of diabetes medication was nausea. (R. at 853). Appellant specifically denied symptoms of neurovascular disease, peripheral neuropathy, and diabetic neuropathy. (R. at 854). The examiner stated that Appellant's diabetic symptoms caused moderate limitations in his ability to perform chores,

shop, engage in recreation, travel, bathe, and dress; mild limitations in exercising and playing sports; and no limitations in feeding, toileting, grooming, or driving. (R. at 855).

A VA psychiatric treatment record from April 2009 indicates that Appellant's memory was intact, and his energy level was fair. (R. at 849 (849-50)). The examiner assigned a global assessment of functioning (GAF) score of 50, indicating serious symptoms. (*Id.*). Later that month Appellant attended a VA mental health examination. (R. at 2034-43). Appellant indicated that his therapy was effective (R. at 2035) but also stated that he was argumentative with his wife and did not like crowds. (R. at 2042). The examiner assigned a GAF score of 53, indicating moderate symptoms. (R. at 2042). In May 2009, a VA regional office (RO) granted service connection for diabetes with a 20% rating and an effective date in December 2008. (R. at 2015 (2015-21)). In that same decision, the Board granted service connection for PTSD, assigning a 30% rating and an effective date in December 2008. (*Id.*). VA treatment records from that same month indicate that Appellant's coronary artery disease was asymptomatic and that his diabetes was borderline controlled. (R. at 840 (838-40)).

The following month, June 2009, Appellant attended another diabetes examination. (R. at 831-37). The examiner noted that Appellant was not restricted in his ability to perform strenuous activities. (R. at 832). The examiner further noted that Appellant denied symptoms of peripheral vascular disease of the lower extremities, neurovascular disease, peripheral neuropathy, and diabetic

neuropathy. (*Id.*). The examiner also stated that Appellant had no limitations in his activities of daily living, to include performing chores, shopping, exercising, playing sports, engaging in recreation, traveling, feeding, bathing, dressing, toileting, grooming, and driving. (R. at 837). And, the examiner noted that Appellant retired because he was eligible by age or duration of work. (*Id.*).

Later that year, in August 2009, Appellant sought service connection for peripheral vascular disease, stating that his hands and feet burned and tingled all the time. (R. at 1977 (1977-78)). In September 2009, Appellant filed a notice of disagreement (NOD) with the May 2009 RO decision's 30% evaluation for PTSD. (R. at 1943-47). Appellant specified that he disagreed with the 30% evaluation for PTSD (R. at 1943), and his representative specified that he was appealing the percentage rating for PTSD. (R. at 1947). Neither Appellant nor his representative referenced the grant of service connection for diabetes. (R. at 1943, 1947). Thus, the RO's May 2009 decision became final with regard to that claim. Also in September 2009, the RO denied, inter alia, service connection for coronary artery disease and peripheral vascular disease of the upper and lower extremities that was claimed as burning and tingling of the hands and feet. (R. at 1933-34 (1933-40)). A VA treatment record from November 2009 noted that Appellant's diabetes was controlled and that his coronary artery disease was asymptomatic. (R. at 1815 (1814-16)).

In January 2010, Appellant sought service connection for peripheral neuropathy of the upper and lower extremities. (R. at 1878-79). That same month,

the RO issued a statement of the case (SOC) that continued the 30% rating for PTSD. (R. at 1900 (1889-1902)). Appellant appealed the denial of an increased rating for PTSD to the Board in March 2010. (R. at 1824-28). Later that year, in June 2010, Appellant attended a VA neuropathy examination. (R. at 1779-82). The examiner diagnosed Appellant with "very mild" peripheral neuropathy of the lower extremities, noting that this condition caused his feet and legs to hurt after walking for five minutes. (R. at 1782). The examiner also recorded Appellant's statement that this condition began in 2007. (R. at 1779). That month, a private physician conducted an EMG study that produced normal results in both legs and the right arm. (R. at 1749). The physician clarified that the results in the lower extremities were consistent with very mild demyelinating peripheral neuropathy that was possibly due to Appellant's diabetes. (*Id.*). The examiner recorded Appellant's statement that he had been experiencing pain and numbness in his hands and feet for the past three years and that he had a one-year history of diabetes. (*Id.*).

In August 2010, the RO issued a decision granting service connection for peripheral neuropathy of the lower extremities, with a 10% rating and an effective date in January 2010. (R. at 1691 (1688-98)). The RO denied service connection for peripheral neuropathy of the upper extremities. (R. at 1692). In June 2011, the RO granted service connection for coronary artery disease, assigning a 30% rating and an effective date in December 2008. (R. at 1616 (1615-22)). In September 2013, Appellant sought TDIU, alleging that all of his service-connected disabilities

prevented him from engaging in a substantially gainful occupation. (R. at 1235 (1235-40)). The RO issued a decision in February 2014 that, inter alia, denied TDIU. (R. at 1185 (1181-89)).

In January 2017, Appellant testified at a Board hearing. (R. at 457-77). Concerning his TDIU claim, he stated that he retired in 2004 and has not worked since that time because of the combination of his service-connected disabilities. (R. at 469). In April 2017, the Board decided Appellant's claims for an increased rating for PTSD and TDIU. (R. at 276-98). In relevant part, the Board denied a rating above 30% for PTSD for the period before May 2012 and denied TDIU. (R. at 298). Appellant appealed to this Court, and, in November 2017, the parties agreed to a joint motion for partial remand (JMPR) of the portion of the April 2017 Board decision that denied TDIU. (R. at 186-90). Specifically, the parties agreed that the Board erred by not considering whether all of Appellant's service-connected disabilities, taken together, caused him to be unable to engage in a substantially gainful occupation. (R. at 186-87). In December 2017, Appellant submitted an affidavit in which he alleged that he stopped working in 2004 as a result of his service-connected disabilities. (R. at 181).

In March 2018,[1] Appellant's counsel procured a private vocational assessment. (R. at 161-72). The private vocational expert opined that Appellant has been unable to engage in substantially gainful employment since 2008 as a

[1] The Board refers to the private vocational report as having been completed in May 2018. (R. at 11). In fact, the report is dated in March 2018, and it was submitted to VA in May 2018. (*See* R. at 161).

result of his service-connected disabilities. (R. at 169). In June 2018, the Board granted TDIU with an effective date in January 2010. (R. at 138 (138-46)). For the period before January 2010, the Board remanded the issue of TDIU for an extra-schedular referral to the Director, Compensation Service. (R. at 144-46). In that regard, the Board noted that, prior to January 2010, Appellant did not have a single disability rated at 60% or higher. (R. at 145; *see* R. at 355 (March 2017 rating decision code sheet)). The RO effectuated the grant of TDIU from January 2010 in October 2018. (R. at 123-28). The following month, the Director, Compensation Service, denied TDIU on an extra-schedular basis. (R. at 117-18). In December 2018, the RO issued a supplemental statement of the case (SSOC), again denying TDIU for the period before January 2010. (R. at 48 (33-50)).

The Board issued the decision on appeal on February 15, 2019. (R at 5-16). The next month, Appellant appealed to this Court.

SUMMARY OF THE ARGUMENT

The Court should affirm the Board's February 15, 2019, decision that denied TDIU for the period before January 21, 2010. The Board did not clearly err in any of its factual findings, it correctly applied the law, and it supported the decision with an adequate statement of reasons or bases.

ARGUMENT

The Board's Factual Findings Are Not Clearly Erroneous, It Correctly Applied the Law, and It Supported the Decision with an Adequate Statement of Reasons or Bases

A total disability rating is warranted when, as a result of his service-connected disabilities, a veteran is unable to secure or follow a substantially gainful occupation. 38 C.F.R. § 4.16. If certain percentage ratings are met, the Board may award TDIU in the first instance. *See* 38 C.F.R. § 4.16(a). Specifically, the Board may grant TDIU when a single disability is ratable at 60% or more or a combination of disabilities are ratable at 70% or more with at least one of them being rated at a minimum of 40%. *Id.* In cases where a claimant is unable to secure and follow a substantially gainful occupation but the percentage standards from § 4.16(a) are not met, the Board must first refer the claim to the Director, Compensation Service, for "extra-schedular" consideration. *See* 38 C.F.R. § 4.16(b). If the Director denies an extra-schedular rating, the Board must then decide whether the evidence shows that the claimant is unemployable because of his service-connected disabilities. *Ray v. Wilkie*, 31 Vet.App. 58, 65 (2019). The Court reviews the Board's conclusions of law de novo. 38 U.S.C. § 7261(a); *Bagby v. Derwinski*, 1 Vet.App. 225, 227 (1991). The Board's determination about whether to grant TDIU after the Director, Compensation Service, denies it on an extra-schedular basis is a finding of fact that the Court reviews under the "clearly erroneous" standard of review. 38 U.S.C. § 7261(a)(4); *Gilbert v. Derwinski*, 1 Vet.App. 49, 53 (1990). The Board must support the decision, to include its

findings about TDIU, with an adequate statement of reasons or bases. *See Allday v. Brown*, 7 Vet.App. 517, 527 (1995).

In denying TDIU before January 2010, the Board considered VA treatment records, as well as the VA examination reports of record. (R. at 8-14 (citing R. at 2370-83 (December 2008 VA treatment record), 853-58, 858-61 (February 2009 VA examination reports), 849-50 (April 2009 VA psychiatric treatment records), 838-40 (May 2009 VA treatment records))). The Board also considered the March 2018 retrospective private vocational report (R. at 161-72) but found it less probative than the contemporaneous VA examination reports and treatment records. (R. at 11). Moreover, the Board discussed Appellant's educational and occupational background when it determined that he was not entitled to TDIU. (R. at 14). The evidence the Board cited provides a plausible basis for the Board's finding that Appellant was not precluded from engaging in substantially gainful employment for the period between 2008 and the grant of TDIU in January 2010, so its finding here is not clearly erroneous. *Gilbert*, 1 Vet.App. at 53. And, because the Board explained the reasons for its decision, it provided an adequate statement of reasons or bases for its denial of TDIU. *See Allday*, 7 Vet.App. at 527.

Appellant first argues that the Board committed legal error when it did not consider the effects of peripheral neuropathy prior to the effective date for the grant of service connection for that condition. (Appellant's Brief at 10-19). In that regard, the Board discussed peripheral neuropathy but concluded that it could not consider its effects on Appellant's employability prior to the effective date for the grant of

service connection in January 2010. (R. at 9; *see* R. at 1691 (August 2010 RO decision granting service connection)). The Board here was correct in declining to consider the effects of neuropathy prior to January 2010, and Appellant's argument is directly contrary to this Court's recent decision in *Delrio v. Wilkie*, __ Vet.App. __; *see* No. 17-4220, 2019 U.S. App. Vet. Claims LEXIS 2233 (Vet. App. Dec. 19, 2019). In *Delrio*, this Court held that "the effective date of TDIU cannot be earlier than the effective date of the award of service connection for disabilities upon which the award of TDIU is based." *Id.* at *31. Thus, there was simply no error in the Board's declining to consider the effects of Appellant's peripheral neuropathy when it denied TDIU. *Id.*

Notwithstanding the holding in *Delrio*, Appellant argues that there is some significance to the fact that the diagnostic code (DC) under which he is rated for diabetes instructs that compensable complications of diabetes should be separately rated. (Appellant's Brief at 11-14); *see* 38 C.F.R. § 4.119, DC 7913, Note 1. The holding in *Delrio* is clear, and Appellant fails to advance any argument as to why it should not apply here. (Appellant's Brief at 11-14). Moreover, in arguing that the December 2008 claim for service connection also encompassed a claim for service connection for peripheral neuropathy (Appellant's Brief at 12-13), Appellant is, in fact, alleging error in the May 2009 RO decision that granted service connection for diabetes. (*See* R. at 2015). But Appellant did not file an NOD with this decision. (*See* R. at 1943, 1947 (September 2009 NOD appealing rating for PTSD)). And he has not alleged or filed a motion for reversal or revision

of the May 2009 RO decision based on clear and unmistakable error (CUE) therein. *See Macklem v. Shinseki*, 24 Vet.App. 63, 68 (2010). Thus, there was no error in the Board's refusal to consider the effects of neuropathy prior to January 2010.

Appellant phrases his next line of arguments in terms of legal error and a failure for the Board to provide an adequate statement of reasons or bases. (Appellant's Brief at 19-28). But Appellant's arguments here amount to a mere disagreement with how the Board weighed the evidence of record and so do not provide a basis for upsetting the decision. (*See id.*); *Deloach v. Shinseki*, 704 F.3d 1370, 1380 (Fed. Cir. 2013) ("We affirm that the evaluation and weighing of evidence are factual determinations committed to the discretion of the factfinder – in this case, the Board."). First, Appellant alleges that the Board provided an inadequate statement of reasons or bases by not considering his physical and mental limitations. (Appellant's Brief at 19-23). But the Board explained its conclusion that Appellant could perform work that was primarily seated, involved little contact with the public and less demanding mental activities, and required only little walking and climbing of stairs. (R. at 14). In this regard, the Board considered Appellant's physical and mental limitations and explained why these did not render him unemployable. (*Id.*). This is all the Board was required to do. *See Ray*, 31 Vet.App. at 74-75.

Appellant next asserts that the Board's discussion of the evidence was flawed because it was "based on irrelevant factors." (Appellant's Brief at 22).

Whether Appellant evaluates a certain piece of evidence as relevant or irrelevant is of no import, as it is the Board – and the Board alone – that has the responsibility of weighing the credibility and probative value of the evidence. *See Deloach*, 704 F.3d at 1380; *D'Aries v. Peake*, 22 Vet.App. 97, 107 (2008). While his argument here is a mere disagreement with the way the Board weighed the evidence, Appellant alludes to an argument that the Board committed legal error when it considered, among other things, evidence that he retired because he had become eligible to retire by virtue of his age and number of years of work. (Appellant's Brief at 22 (citing R. at 9)). Appellant correctly notes that the Board must consider whether a claimant is unemployable without regard to his age. (Appellant's Brief at 22 (citing 38 C.F.R. § 3.341(a)). But here, the Board did not base its denial on the fact that Appellant had reached retirement age. (R. at 9). Rather, the Board properly considered that there was conflicting evidence in the record – some evidence suggested that Appellant retired because of his service-connected heart condition and other evidence suggested that he retired for other reasons. (R. at 9). The Board was well within its rights to discuss and weigh the probative value of this conflicting evidence. *See Deloach*, 704 F.3d at 1380. Moreover, the Board did exactly what Appellant alleges it failed to do by considering whether his limitations would render him unemployable. (R. at 14).

Appellant next announces that the Board erred in discounting the probative value of the March 2018 private examiner's report "because the treatment notes were not at odds with the vocational expert's findings." (Appellant's Brief at 23).

Appellant's argument here is nothing more than an invitation for the Court to re-weigh the evidence of record, something it cannot do. *Singleton v. Shinseki*, 23 Vet.App. 376, 381 (2010). And Appellant's assertion that the Board did not explain what evidence was inconsistent with his current assertions that his service-connected disabilities caused him to be unable to work prior to January 2010 is baseless. (Appellant's Brief at 24). Specifically, the Board noted that cardiac treatment records from September 2008 did not show the degree of limitation that Appellant now alleges. (R. at 10 (referencing R. at 2181)). The Board also referenced the June 2009 VA examiner's conclusion that Appellant was not restricted in his ability to perform strenuous activities (R. at 832) and that his diabetes did not prevent activities such as doing chores, shopping, or exercising (R. at 837). (R. at 11). And the Board noted that a VA provider's November 2009 treatment record indicated that Appellant's heart condition was asymptomatic and his diabetes was controlled. (R. at 11 (citing R. at 1815)).

Moreover, the Board considered the February 2009 VA examiner's conclusion that Appellant was more limited by his service-connected heart condition than the other records suggested. (R. at 11 (citing R. at 859)). The Board's reasoning here is clear on the face of the decision. (R. at 10-11). In the same vein, the Board also considered evidence that Appellant had significant psychological symptoms but noted that his symptoms appeared to respond to treatment. (R. at 12-13 (referencing R. at 849 (April 2009 VA treatment record), 2034-43 (April 2009 VA examination report))). Thus, Appellant's allegation that

the Board failed to explain its findings that the record was inconsistent with his allegations about the extent of his limitations is simply false. Finally, Appellant's argument that the Board failed to explain evidence that he had difficulty dealing with customers, would make mistakes at work, and was often tired (Appellant's Brief at 24) is entirely unpersuasive. In fact, the Board specifically considered these symptoms but explained that, even with these symptoms, Appellant would not be precluded from employment. (R. at 14).

Appellant bears the burden of demonstrating prejudicial error on appeal, but, in this case, he has not established that the Board committed any error, much less one warranting remand. *See* 38 U.S.C. § 7261(b)(2) (directing that the Court is required to "take due account of the rule of prejudicial error"); *Shinseki v. Sanders*, 556 U.S. 396, 406 (2009); *Hilkert v. West*, 12 Vet.App. 145, 151 (1999) (en banc) (holding that appellant bears burden of demonstrating error on appeal), *aff'd*, 232 F.3d 908 (Fed. Cir. 2000); *Marciniak v. Brown*, 10 Vet.App. 198, 201 (1997) (holding that the appellant bears the burden of demonstrating prejudice on appeal and that remand is unnecessary "[i]n the absence of demonstrated prejudice"). Because Appellant has not established Board error, the Court should affirm the Board's decision.

CONCLUSION

In light of the foregoing, the Court should affirm the Board's February 15, 2019, decision.

Respectfully submitted,

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